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Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
PRESIDENTIAL EBOLA TASK FORCE 17
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°151 /MVE-BDBV/12/10/2026

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HIGHLIGHTS (last 24 hours)

- Seventy-nine (79) new confirmed cases of Ebola Virus Disease (EVD), including 18 community deaths, have been reported. These cases originated from the provinces of Ituri (40 cases), North Kivi (27 cases), Haut Uele (8) and Tshopo (4).
  In addition, 11 deaths were recorded in the CTEs.
- A total of 419 new samples were analyzed; 79 of these samples tested positive for the Bundibugyo virus, representing a positivity rate of 18.9%.

ACCUMULATIVE CAS	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
9144	4 410	48,2%	899	2 418	77,6%

Affected provinces and affected health zones

7 Provinces affected

Uele, Ituri, North Kivu, South Kivu, Tshopo, Lower Uele and Sud Ubangi

65 Health Zones affected out of 167 (38,9%)

Summary of key points from the last 24 hours

In the last 24 hours, **79 new confirmed cases** of Ebola virus disease (EVD) have been reported in four provinces: **Ituri (40 cases), North Kivu (27 cases), Haut-Uélé (8 cases) and Tshopo (4)**

In addition, **11** deaths were recorded in the CTEs (8 in Ituri and 3 in North Kivu, This brings the total number of confirmed deaths for the period in question to 29.

In addition, 29 patients (13 in Ituri, 13 in North Kivu, 2 in Haut-Uélé and 1 in Tshopo) were declared cured after obtaining two negative results in the control test for Ebola virus disease.

Since the start of the epidemic, the DRC has recorded **9,144** confirmed cases and **4,410** deaths, representing a case fatality rate of **48.2%.** Ituri remains the epicenter of the epidemic, accounting for **73.6** % of cumulative confirmed cases and **50.6%** of new confirmed cases reported in the last 24 hours.

65 out of 167 health zones (38.9%), spread across seven provinces, have been affected since the start of the epidemic.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of October 12, 2026.

Province	New cases confirmed (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	40	6734	3 117	46,3%	28/36 (77,8 %)
North Kivu	27	1938	1 102	56,9%	17/34 (50,0 %)
Haut-Uélé	8	391	163	41,7%	7/13 (53,8 %)
Boss	4	63	22	34,9%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	0	13	4	30,8%	4/11 (36,4%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Total	**79**	**9 144**	**4 410**	**48,2%**	**65/167 (38,9%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 12, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**6734**	**3117**	**46,3%**	**40**	**10**	**8**	**18**
Give	11	1	9,1%				0
Understand	9	6	66,7%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	214	48	22,4%	0	0	1	1
have	2	2	100,0%				0
Bunia	1848	614	33,2%	16	1	1	2
Ladies	40	9	22,5%				0
Drodro	15	7	46,7%				0
Fireworks	84	31	36,9%				0
Gethy	11	7	63,6%				0
Flounder	2	0	0,0%				0
Kilo	38	12	31,6%				0
Team	182	109	59,9%	3	2	0	2
Liter	304	155	51,0%	2	0	1	1
Logo	14	8	57,1%			1	1
Fight	36	9	25,0%	1	0	0	0
distributed	4	1	25,0%				0
Washing	45	18	40,0%	2	0	0	0
Darkness	269	139	51,7%	11	7	0	7
Mangala	370	161	43,5%				0
Mongbwalu	712	342	48,0%	0	0	1	1
Nanny	327	183	56,0%				0
Nizi	816	291	35,7%	3	0	1	1
Legumes	133	39	29,3%				0
Jungle	18	5	27,8%				0

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Rwampara	1134	395	34,8%	2	0	2	2
Tchomia	76	25	32,9%				0
A valves	THAT	493	THAT				0
North Kivu	**1938**	**1102**	**56,9%**	**27**	**7**	**3**	**10**
Me	471	316	67,1%	13	6	1	7
Farm	23	7	30,4%				0
Butembo	297	185	62,3%				0
Ten	1	0	0,0%				0
Kalunguta	72	27	37,5%	2	1	0	1
Cut	630	383	60,8%	2	0	0	0
Source	3	2	66,7%				0
Kyondo	38	20	52,6%	1	0	0	0
Lubero	28	13	46,4%	0	0	1	1
Be anxious.	64	22	34,4%	1	0	0	0
Alimbong	5	3	60,0%				0
Manguredjipa	15	5	33,3%				0
Masereka	37	12	32,4%	1	0	0	0
The museums	147	69	46,9%	4	0	0	0
Mutwanga	23	5	21,7%			1	1
Oicha	45	20	44,4%	3	0	0	0
Vuh	39	13	33,3%				0
Upper Uele	**391**	**163**	**41,7%**	**8**	**1**	**0**	**1**
Government Mangbetu	44	19	43,2%				0
Dumb	1	1	100,0%	0	0	0	0
Gombari	3	1	33,3%				0
Mathematics	104	46	44,2%	2	0	0	0
Lining	100	49	49,0%	0	1	0	1
Beans	5	2	40,0%				0
Ordinary	134	45	33,6%	6	0	0	0
Boss	**63**	**22**	**34,9%**	4	0	0	0
Bafwasende	10	1	10,0%	3	0	0	0
Kabondo	7	1	14,3%	1	0	0	0
Lubunga	1	0	0,0%				0
Maquis - Kisangani	32	12	37,5%				0
Mangobo	10	7	70,0%				0
Boss	2	0	0,0%				0
Wanie -Rukula	1	1	100,0%				0
South Kivu	**3**	**1**	**33,3%**	0	0	0	0
Miti -Murhesa	3	1	33,3%				0
Bas Uélé	**13**	**4**	**30,8%**	0	0	0	0
Blind	1	1	100,0%				0
Viadana	6	1	16,7%				0
Claim	1	0	0,0%				0
Walk	5	2	40,0%				0
Sud Ubangi	**2**	**1**	**50,0%**	0	0	0	0
Hair	2	1	50,0%				0
Total	**9144**	**4410**	**48,2%**	**79**	**18**	**11**	**29**

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*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA : Not Applicable.

Table 3. Status of alerts notified as of October 12, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	142	7	149	27	7	64	0	34	12
Bas Uélé	169	1	170	21	1	148	0	22	0
Ituri	1 265	99	1364	177	99	1086	0	250	131
North Kivu	1 096	46	1142	138	45	958	1	183	118
South Bank ND		ND	ND	ND	ND	ND	ND	ND	ND
Boss	272	3	275	4	3	268	0	7	1
Total General	**2944**	**156**	**3100**	**367**	**155**	**2524**	**1**	**496**	**262**

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

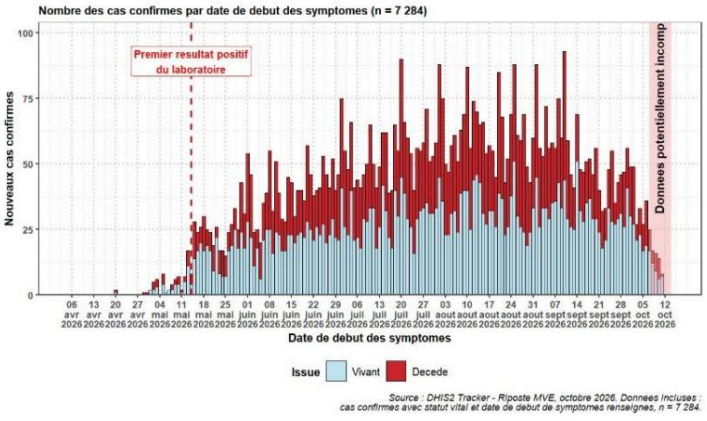
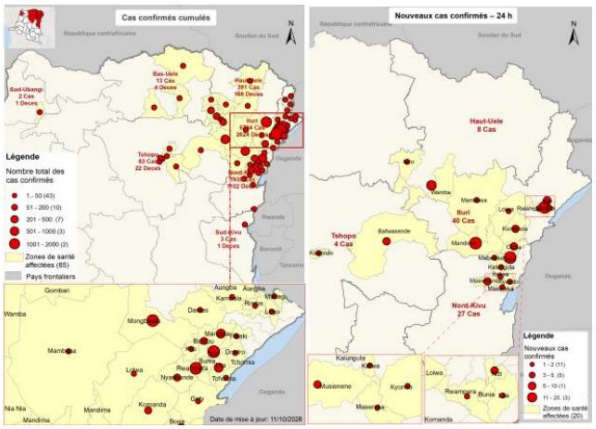


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1.COORDINATION

1.1. Main Actions

• Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

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As of October 12, 2026, 3,098 alerts had been reported in the five provinces that submitted their data (Ituri, North Kivu, Haut-Uélé, Bas-Uélé, and Tshopo). Of these, 3,047 (98.3%) were verified, with 522 (16.8%) validated as suspected cases. Of these 522 suspected cases, 496 (95.0%) were investigated, with 262 (52.8%) transferred to COVID-19 treatment centers (CT/CTE). 26 suspected cases in Ituri were not investigated, and none of the 22 suspected cases in Bas-Uélé were transferred. South Ubangi did not submit its data.

Of the **32,811 contacts being monitored** in the five provinces that submitted their data, **25,469 contacts** were seen in the last 24 hours, representing a follow-up rate of **77.6** %, down from the previous day (79.5%) and below the required threshold of 95.0%. This rate was **89.4% (1,344/1,504)** in Haut-Uélé, **88.0% (470/534)** in Bas-Uélé, **86.2% (834/967)** in Tshopo, **82.3% (11,045/13,415)** in Ituri, and **71.8% (11,776/16,391)** in Nord-Kivu. No data was submitted by Sud-Ubangi.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

On October 12, seven (7) alerts were notified to the PoE/PoC: four (4) in Ituri, one (1) in North Kivu, one (1) in South Kivu, and one (1) in Tanganyika. All alerts were investigated and then transferred to the (CTE).

Table 4. Status of key PoE/PoC indicators by province, as of October 12, 2026

Indicators	Ituri	North Kivu	South Kivu	Boss	Haut-Uélé	Global
Report completeness	93,4% (57/61)	90,5% (19/21)	55,6% (5/9)	44,4% (8/18)	90,0% (9/10)	82,4% (98/119)
% of functional strategic PoE/PoC	100%	100%	ND	100%	90%	ND
Number of people who have switched to PoE/PoC	148 545	94 720	6 835	9 225	17 740	277 065
% of travelers screened	98,0%	97,2%	98,0%	97,0%	99,3%	97,8%
% of travelers who washed their hands	97,4%	97,2%	98,0%	96,4%	99,1%	97,4%
% of travelers made aware	99,8%	97,2%	100,0%	100,0%	100,0%	98,9%
Number of alerts notified	4	1	1	0	1	7
Number of live alerts validated (suspected cases)	3	0	0	0	0	3
Number of bodies recovered today	1	1	1	0	1	4
% of suspected cases transferred to the CTE/CT	33,3%	THAT	THAT	THAT	THAT	33,3%
% of lifeless bodies swabed/secured	100,0%	100,0%	0,0%	THAT	100,0%	75,0%
% of suspected cases investigated within 4 h	100,0%	100,0%	0,0%	THAT	100,0%	85,7%

2.2.2. Challenges

In Ituri, the delay in September salary payments, the low encoding of alerts in the DHIS2 Tracker, and the insufficient mobility for supervision in the Rwampara and Bunia health zones persist; in North Kivu, completeness has fallen to 90.5% (19/21) and the Butuhe PoC remains non-functional since September 26, 2026. In Haut-Uélé, the Mutowamba PoC did not report due to a lack of network connection; in Bas-Uélé, only 19 out of 41 notification sites are functional (46.3%), none of the 11 PoEs have been reinforced, and the service providers for the PK12 Isiro PoC (Viadana health zone) resigned due to two months of unpaid bills; In Tshopo, the completion rate was only 44.4% (8/18), with ten private ports not operating on Sunday, and 2.9% of travelers refusing screening.

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2.3. Laboratory

2.3.1. Main actions

- **Ituri: 40 new positive results** (30 living and 10 dead) out of 227 samples received and analyzed **(positivity: 17.6%).**
- **North Kivu: 27 new positive results** (20 living and 7 dead) out of 141 samples received and analyzed **(positivity: 19.1%).**
- **Haut-Uélé: 8 new positive results** (7 living and 1 death) **out of 36 samples received and analyzed (positivity: 22.2%).**
- **Tshopo: 4 new positive results (4 live) 12** samples received **and** analyzed **(positive) : 33,3 %).**
- **Bas-Uélé: 3 new samples** received and analyzed **(all live),** all tested negative

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 26 out of 55 rings were opened (47.3%), 40 out of 51 households were decontaminated (78.4%), and 7 FOSA decontaminated out of 7 (100%). Out of 113 EDS alerts (including 5 postponements from the previous day), 80 EDS were carried out (70.8%) and 76 bodies swabed out of 102.

- **North Kivu:** 13 out of 22 infection control rings were opened (59.1%), with a gap in Katwa, Kalunguta, Mabalako, and Manguredjipa; 13 out of 22 infection control centers (59.1%) were decontaminated, and 3 out of 15 households (20.0%) were decontaminated. None of the decontaminated households received infection control kits. 5 infection prevention and control (IPC) assessments (scorecard) were conducted with an average score of 43.1%. 463 healthcare providers were briefed in 164 monitored infection control centers, and no people with primary AIDS (PLAs) were infected among the 28 confirmed cases on October 11 (0/28). Of 59 death alerts and 45 reports from the previous day, 63 death reports were carried out and 59 bodies were swabed, with 4 failures (refusal of the body bag in Kyondo and Mutwanga) and 41 reports postponed to the following day.

- **Haut-Uélé:** 2 out of 2 rings were opened (100%), 5 out of 5 households were decontaminated (100%), and 3 out of 3 ESS (100%) were completed in Isiro, Rungu, and Pawa; 3 ESS were assessed (45.0% at Notre-Dame de Neisu Hospital, 29.0% at the CS, and 25.8% at the Bafwabaka Hospital), and 53 PPLs (People Living with Distress) were briefed (10 in Isiro, 20 in Pawa, 18 in Wamba, and 5 in Boma Mangbetu). Of 9 EDS (Emergency Disposal) alerts, 6 EDS were carried out (66.7%), and 7 bodies were swabed without failure. 2 EDS remain in progress in Isiro due to a lack of graves and coffins.

- **Tshopo:** 17 out of 17 ESS (Emergency Situations) were monitored in 5 health zones and 201 service providers were briefed; the University Clinics, Alwaleed Hospital, the Imani Health Center, and the Makiso-Kisangani Health Zone (around the 59th case), as well as the Saint Francis of Assisi Health Center (Kabondo, scorecard of 14.2%), were decontaminated. The cumulative total reached 52 out of 52 rings opened (100%) and 35 out of 38 households decontaminated (92.1%); out of 3 EDS (Emergency Discharge Summary) alerts at the Ebola Treatment Center, 3 bodies were swabs and 2 EDS were carried out

- Bas-Uélé: 3 out of 4 infection control rings have been opened (75.0%) and 3 out of 4 households decontaminated (75.0%) in Ganga (Sopi and Bambesa health centers), as well as the Azange primary school and 3 latrines; the household and the community health center (CHC) in Dakwa remain to be decontaminated due to a lack of equipment. 5 out of 5 CHCs were monitored in Buta and Ganga, 31 healthcare providers and 7 hygiene workers were briefed, and the WHO provided infection prevention and control (IPC) supplies to the Buta General Referral Hospital, the Rubi Lisanga Hospital, and the Mobenge Health Center; 1 body was swabed (Congo health center), without a complete blood count (CBC), with 1 CBC gap in Buta.

2.4.2. Challenges

- In Tshopo, the lack of inputs and trained providers in IPC and the breakdown of the incinerator at the Makiso-Kisangani General Referral Hospital persist; only 2 out of 104 health establishments (1.9%) exceed 80% in the IPC score and 20 health workers are among the 63 confirmed cases in the province.

- In North Kivu, the gap in ring openings (13/22; 59.1%), the low household decontamination (3/15; 20.0%), the lack of provision of kits to decontaminated households, community resistance, the lack of trained decontamination teams and mobility (Katwa, Biena, Masereka, Kyondo, Mabalako and Vuhovi) and the high-risk exposure of 9 out of 32 assessed PPLs persist.

- In Bas-Uélé, 16 out of 20 priority ESS (80.0%) do not have functional sorting, waste management areas, hygienic latrines, garbage cans and DLM are insufficient, and the low reporting of community death alerts and low community engagement limit EDS.

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2.5. Holistic care

2.5.1. Main actions

In Ituri, 100 new admissions (all suspected cases, including 6 in the advanced stage and 1 patient with HIV in Rwampara) were recorded, compared to 75 discharges, including 13 recoveries, 47 negative cases, 11 deaths (8 confirmed), and 4 discharges without medical advice. Thus, 382 patients (127 confirmed and 255 suspected) are hospitalized, representing an occupancy rate of 36.0%, including 38 critically ill patients; 28 patients were evacuated by ambulance, and 358 out of 368 patients (97.3%) received a hot meal. The total number of recoveries in the province has reached 1,908.

• In North Kivu, 86 new admissions (5 confirmed and 81 suspected) were recorded, compared to 82 discharges, including 13 recoveries, 63 non-cases, and 6 deaths. Thus, 445 patients (176 confirmed and 269 suspected, including 52 children under 5 years old) are hospitalized: 333 in Ebola Treatment Centers (ETCs) out of 374 beds, representing an occupancy rate of 89.0%, and 112 (25.2%) in other healthcare facilities (ESSs); 6 patients are in critical condition (Beni ETC and Kitatumba ICU), and all 21 ambulance trips were completed.

In Haut-Uélé, six new admissions (one confirmed case in Isiro and five suspected cases in Pawa and Wamba) were recorded, compared to two recoveries and four negative cases in Boma Mangbetu, two confirmed deaths (Pawa and Wamba), and one suspected death in Wamba. Currently, 45 patients (17 confirmed and 28 suspected) are hospitalized out of 169 beds, representing an occupancy rate of 26.6%. One confirmed case is in critical condition in Pawa, and two suspected cases are in critical condition (Pawa and Wamba). Forty-three patients, 52 caregivers, and 70 people with disabilities received hot meals, and two patients were transported by ambulance. The province's total number of recoveries has reached 157.

• In Tshopo, 3 new admissions (2 confirmed cases transferred from the treatment center at Bafwasende General Referral Hospital and 1 suspected case) were recorded, compared to 1 discharge, a recovery. Thus, 13 patients (7 confirmed and 6 suspected) are hospitalized out of 36 beds, representing an occupancy rate of 36.1%, including 1 patient in serious condition; the 54th confirmed case has still not been transferred to the treatment center. Since the beginning of the outbreak, 443 people have received the experimental treatment, including 41 confirmed cases, and 402 have received post-exposure prophylaxis.

• In Bas-Uélé, no new confirmed cases were admitted; 11 new suspected cases were admitted to isolation sites (Viadana 3, Bondo 3, Ganga 2, Dakwa 1, Likati 1, and Titule 1), while 3 non-cases were discharged, with no recoveries or deaths. Thus, 14 patients (3 confirmed and 11 suspected) remain in isolation, while 15 verified cases are not isolated and remain in the community; monitoring of the child in isolation in Dingila and the confirmed case in Dakwa continues.

2.5.2. Challenges

In Ituri, the low capacity of some CTE/CT (Mambasa, Kilo, Lolwa, Mangala, Mandima and Biakato), the care of 38 critical patients, the lack of food for 10 patients in the Mandima health zone and the low reporting of ambulance movements (18/33) remain worrying.

In North Kivu, the saturation of suspected beds (160/165; 97.0%), the care of 112 patients (25.2%) outside of CTE/CT and an ambulance response capacity limited to 50% (14/28) remain worrying.
In Haut-Uélé, poor network coverage in some health zones, demotivation among some staff, insufficient nutritional inputs, and inadequate water and electricity supplies to some treatment centers remain challenges to be addressed.

In Tshopo, the lack of resuscitation equipment at the CTE and of staff for post-exposure prophylaxis weakens the care provided.

In Bas-Uélé, the lack of appropriate nutritional care for isolated patients, of ambulances for the transfer of patients and of standardized CTE, still under construction at the HGR of Buta, remains a major operational challenge.

2.6. Vaccination

2.6.1. Main actions

• In Haut-Uélé, the cumulative total reached 470 PPL vaccinated out of 1,060 pre-registered (44.3%), including 249 women and 221 men; 470 doses out of 600 received were used and 130 returned, without loss, and the pre-registration of PPLs was launched in the ZS of Pawa, Boma Mangbetu and Wamba.

• In Ituri, 5,900 doses of Ervebo vaccine were received from the national EPI and vaccination of PPL continued with the support of MSF: 88 people were vaccinated on October 12 (30 women and 58 men), bringing the cumulative total to 938 vaccinated (395 women and 543 men), including 11 pregnant women and 25 breastfeeding women, with 1 AEFI notified; research on the effectiveness of the Ervebo vaccine against the BDBV strain and the study on the impact of Ebola virus disease on the use of sexual and reproductive health services continued.

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2.6.2. Challenges

In Ituri, the target for PPL and TPL remains to be redefined for the calculation of vaccination coverage (93.8%; 938/1,000).

In Bas-Uélé, the provincial stock of Ervebo vaccine is out of stock (0 doses), the start-up of the UCCs is not effective and the funds for vaccination are still not available; vaccination coverage remains at 11.4% (1,500/13,202).

- **In North Kivu,** indicators of continuity of essential services were analyzed in the health zones of Butembo, Beni, Katwa and Musienene for the purpose of supervision.
- **In Bas-Uélé,** the Dakwa General Referral Hospital is partially operational, with 13 staff members present out of 36 planned (36.1%), a decrease in attendance and the absence of a water point and a waste area.

2.7. Continuity of Care

2.7.1. Main actions

- In Tshopo, the REGIDESO CM assessment showed the absence of an effective triage area, non-operational maternity and PN services and no agents vaccinated against Ebola (0/13), despite a functional incinerator; the recovery plan for CAKIS was developed, and the monitoring of PPL at the CTM and CTE as well as that of recovered patients with comorbidities at the Makiso-Kisangani General Referral Hospital continued.
- **In North Kivu,** the baseline assessment of the Beni General Referral Hospital on the continuity of essential healthcare services was completed.
- **In Haut-Uélé,** the evaluation of CH Mayogo (ZS of Isiro) highlighted low use of services due to fear of contamination, missed appointments for routine vaccination and monitoring of chronic diseases, a non-functional borehole due to lack of energy and insufficient latrines and showers; the monitoring of 3 PPL at the CTE of Wamba and Boma Mangbetu continued.
- **In Bas-Uélé,** no data has been transmitted by the continuity of care pillar in the last 24 hours.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- In Ituri, 15,670 home visits reached 50,130 people; 345 educational talks brought together 5,502 people, 114 mass awareness campaigns reached 7,555 people, 2 community debates reached 112 people and 10 advocacy sessions reached 44 authorities and leaders; 8 spots were broadcast on 36 radio stations and 2 programs were organized.
  In total, 209 community alerts, including 84 deaths, were reported; 45 out of 58 feedback requests (77.6%) and 4 out of 6 incidents (66.7%) were resolved within 48 hours, and the CREC facilitated the pre-listing of 264 contacts around 10 confirmed cases, the decontamination of 25 out of 25 households (100%), 24 out of 28 EDS (85.7%), and the referral of 24 out of 27 suspected cases to the CTE. The completeness of CREC reports is only 35.7% (10/28 ZS).

- In Bas-Uélé, 332 home visits reached 1,010 people, and 13 educational talks were held with 75 people in the Agri Uele, Bambili, Sopi, and Dembia (Ganga) districts; 798 students at the Ngelu Primary School (Monga) and 452 students and 12 teachers in Dakwa were sensitized, and 50 community health workers, community health activists, and traditional healers were briefed in Dakwa. All 20 community alerts reported in Ganga and Buta were addressed, while the community radio station in Dingila has been out of service for several months.

- In Tshopo, 1,512 home visits reached 6,770 people, 9 mass awareness campaigns reached 2,959 people, and 3 educational talks reached 53 people, notably in schools, with motorcycle taxi drivers from UMTRD and AJTRD, and at the Dokolo port; the Governor addressed the population on Ebola, and the 48th case was discharged from the Meteorological Treatment Center (CTE Météo) having recovered. All 160 community alerts were addressed, and the 3 feedback received was resolved (100%), including rumors about protection through alcohol and palm flowers, while the one reported incident (0/1) remains unresolved; the CREC supported the follow-up of 80 contacts related to the 48th and 51st cases and 5 decontamination procedures in Makiso-Kisangani.

- In Haut-Uélé, 4 mass awareness sessions reached 4,105 people in schools and the community of Wamba, 9 educational talks reached 98 people in Isiro and 15 home visits reached 86 people in Wamba, totaling 4,289 people, with continued intensification of awareness by the Vice-Governor in Wamba; 100 community alerts and 5 death alerts were notified, all 9 feedback received were resolved (100%), and 43 family members linked to two suspected deaths were sensitized to contact tracing, decontamination, and EDS in the Tely and Gamba health districts (Isiro).

- In North Kivu, 13,133 home visits reached 40,652 people, 279 educational talks reached 6,939 people, 79 mass communications reached 8,069 people, and 5 briefings reached 70 people; 435 community alerts were notified out of 650 expected (66.9%), 26 out of 32 feedback requests were received (81.3%), and 9 out of 11 incidents (81.8%) were resolved within 48 hours. 2,025 out of 4,750 people (42.6%) were engaged in

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community actions, a plan to strengthen CREC in schools has started in Lubero and 67 contacts have been listed with the support of CREC in Katwa (38) and Manguredjipa (29).

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- In Ituri, 24 of the last 56 confirmed cases (42.9%), 84 confirmed cases under follow-up out of 105 (80.0%), 50 new suspected cases out of 95 (52.6%), and 111 suspected cases under follow-up out of 252 (44.0%) benefited from SMSPS interventions. The teams supported 255 caregivers and 70 people living with HIV/AIDS (PLHIV), monitored 24 out of 29 recoveries (82.8%) in the community and 15 out of 15 children in daycare, provided support to 19 out of 26 separated and orphaned households, reintegrated 9 out of 13 recoveries, and announced 48 results, including 24 positive ones; 102 children are currently being monitored.

- In North Kivu, 25 of the last 28 confirmed cases (89.3%), 312 confirmed cases under follow-up, 134 new suspected cases and 276 suspected cases under follow-up benefited from SMSPS interventions; the teams accompanied 98 households and 42 households of separated and orphaned children out of 50 (84.0%), supported 89 PPLs, followed up with 35 children in the community and 5 out of 6 children in daycare, reintegrated 7 recovered individuals, followed up with 12 recovered individuals in the community and announced 102 results, including 28 positive results; 384 people benefited from psychoeducation with the support of UNICEF.

- In Tshopo, 5 out of 5 confirmed cases under follow-up (100%), 2 out of 2 new suspects and 7 out of 7 suspects under follow-up benefited from SMSPS interventions in 5 out of 7 health zones that notified (71.4%); 657 people, including 456 students from EP Baikuba (Wanie-Rukula), benefited from psychoeducation, 2 results, all negative, were announced and 2 out of 3 resistances were broken.

- In Haut-Uélé, 3 new confirmed cases, 11 confirmed cases under follow-up, 5 new suspected cases and 18 suspected cases under follow-up benefited from SMSPS interventions in the CTEs of Isiro, Pawa and Boma Mangbetu (Wamba not having reported); the teams supported 47 caregivers, 54 PPLs, 2 children separated in community and 5 household members and announced 24 results, including 4 positive ones; in Bas-Uélé, no SMSPS interventions were reported in the last 24 hours.

2.9.2. Challenges

- In Ituri, only 14 out of 28 affected health zones are covered (50.0%), the Mandima health zone, which concentrates 32% of the cases notified today, is not covered and inter-pillar collaboration needs to be strengthened for interventions around cases in Bunia and Mambasa.

- **In Bas-Uélé,** SMSPS coverage remains embryonic, with activities having only just begun in the ZS from Viadana.

- **In Haut-Uélé,** the mobility of community health workers remains difficult in Wamba and Boma Mangbetu, nutritional kits are lacking for weaned infants in Wamba and Isiro, the community health worker lacks computer equipment, and data collection remains weak in Boma Mangbetu and Pawa

- In Tshopo, there is a need to train the APS (Auxiliary Police Officers) of the Wanie-Rukula Health Zone and an insufficient number of units communication and motorcycles for the movement of the pillar.

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2.10. Logistics

2.10.1. Main actions.

Ensuring the mobility of all field teams in the affected provinces; in Tshopo, monitoring of the construction of the Ebola Treatment Centers (ETCs) in Bafwasende and Kongakonga, the extension of the Meteorological ETC and the rehabilitation of the isolation center in Makiso continued and vaccines were deployed in the health zones of Kabondo, Makiso-Kisangani and Tshopo; in Bas-Uélé, the construction of the ETC at the Buta General Referral Hospital continues and surveillance teams have been deployed in Ganga and Viadana; in North Kivu, the opening of the Musienene ETC is planned for October 15.

2.11. Security

2.12. Main actions

In North Kivu, public order has been restored in the Ngongolio AS, where the population was blocking the road to the PCI team, without any damage recorded; the teams were escorted for the burial of two confirmed cases at the Bundji cemetery (morgues of Ngalika and the HGR of Beni), the decontamination of the CM Mingu ni Baba (Ngongolio), the CM Santé Plus (Malepe) and a household, as well as for the swab of lifeless bodies at the CH Anglican, in Wayene (Katwa), at the HGR Mama Rose and in the Kasesa cell.

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2.13. PSEA (Prevention of Sexual Exploitation and Abuse)

2.13.1. Main actions

- In Ituri, the PSEAH training held in Bunia with the support of the IMC was closed with the presentation of certificates to the successful candidates and 18 traders from the Chem-Chem market were sensitized; in Tshopo, 85 actors from six pillars of the response (52 women and 33 men) were briefed on PSEAH with the support of the WHO and signed the code of good conduct, and 641 people were sensitized in Kabondo, Makiso-Kisangani and Mangobo; in Haut-Uélé, 49 people were sensitized on PSEAH and the toll-free numbers 495555 and 151 in Wamba (heads of women's associations and Opéa village) and at the Boma Mangbetu General Referral Hospital (nurses).

- In North Kivu, 249 people were sensitized on the PSEA and the reporting numbers 151 and 495555 during 4 community sessions in the health zones of Beni (98 people, AS Benengule and Paida) and Butembo (151 people, AS Ngengere and Makasi), and a reminder message on the code of good conduct was sent to 27 members of the coordination teams.

2.13.2. Challenges

At the national level, only 1 out of 7 DPS (14.3%) reported its PSEA activities to the national coordination (North Kivu); in Tshopo, coverage remains low, with 7 out of 23 health zones covered and 85 staff trained out of 270 (31.5%), while the proportion of health zones aware of the reporting mechanism remains low in Ituri (36.1%; 13/36), North Kivu (17.6%; 6/34) and Haut-Uélé (46.2%; 6/13).

MESSAGES CLES

Over the past 24 hours, 496 out of 522 suspected cases (95.0%) have been investigated in the provinces that submitted their data. However, 26 suspected cases remain uninvestigated in Ituri. Furthermore, the contact tracing rate has decreased from 79.5% the previous day to 77.6%, remaining below the required threshold of 95.0%. This situation necessitates strengthening epidemiological surveillance and contact tracing to limit the risk of community transmission.

Maintain a level of vigilance regarding epidemiological surveillance aspects, consolidate IPC and CREC actions at the community level while providing quality care in dedicated care structures with adequate reception capacities.

TIMELINE OF KEY EVENTS

ÿ From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

ÿ Between May 17 and 18, 2026, national authorities , with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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